TrakCare Lab Web Results Viewer

LABORATORY REPORT

PATIENT:

Mabotsebane MATLALA
31/12/80 (45y) Sex F

LAB NUMBER: **LH M0000621**

Sample Ref: AKBA7503NOF
Collected: 17/03/2026 12:21
Received: 17/03/2026 13:31
1st Print: 19/03/2026 11:22
Reprint: 19/03/2026 15:00

REPORT TO:

Dr N. KHAYIYANA
Female Medical Ward
Bheki Mlangeni Hospital
PO Box 731
Kwa-xuma
Jabulani
Gauteng
1868

Patient Location: Bheki Mlangeni Hospital, Female Medical Ward
Hospital Number: **11162673**

DISCLAIMER: If the date or time of collection is not included on the request form or the sample does not meet our laboratory requirement for collection, transport and processing, caution should be exercised when interpreting results as their accuracy cannot be guaranteed. Absent clinical details may affect the laboratory's interpretation of results.

FOR ENQUIRIES AND FOLLOW-UP TESTS, PLEASE QUOTE PATIENT'S MRN NUMBER MRN194769552

CHEMICAL PATHOLOGY

Specimen received: Clotted blood, EDTA blood
Tests requested: Na, K, Cl, Bicarb, Urea, Creat, HbA1c, Ca, Mg, PO4, TP, Alb, T bili C bili, ALT, AST, ALP, GGT, Lipids, CRP, Vit B12, TSH, FT4, Indices

Blood chemistry:

Sodium	136	mmol/L	136 - 145
Potassium	4.2	mmol/L	3.5 - 5.1
Chloride	106	mmol/L	98 - 107
Bicarbonate	**16 L**	mmol/L	23 - 29
Anion gap	**18 H**	mmol/L	9 - 16
Urea	3.6	mmol/L	2.1 - 7.1

Creatinine and estimated GFR:

Creatinine	**102 H**	umol/L	49 - 90
eGFR (MDRD formula)	51	mL/min/1.73 m^2	
eGFR (CKD-EPI formula)	57	mL/min/1.73 m^2	

MDRD-derived estimation of GFR may significantly underestimate true GFR in patients with GFR > 60 mL/min/1.73m^2. It may also be unreliable in the case of: age <18 years or >70 years; pregnancy; serious co-morbid conditions; acute renal failure; extremes of body habitus/unusual diet; gross oedema. The MDRD-eGFR used here does not employ an ethnic factor for race.

Estimation of the GFR using the CKD-EPI formula is more accurate than the MDRD formula at GFR > 60 mL/min/1.73 m^2, and can therefore be used to detect patients with mild renal impairment. It can be used to grade severity of renal impairment using the KDIGO 2012 recommendations. The CKD-EPI formula used here does not employ a correction factor for race.

Blood chemistry:

Calcium	**2.08 L**	mmol/L	2.15 - 2.50
Magnesium	0.70	mmol/L	0.63 - 1.05

Mabotsebane MATLALA

LAB NUMBER: **LH M0000621**
Collected: 17/03/2026 12:21
Printed: 19/03/2026 15:00

Bheki Mlangeni Hospital, Female Medical Ward, Hospital Number 11162673

Blood chemistry:<Continued>

Inorganic phosphate	**0.72 L**	mmol/L	0.78 - 1.42

Liver function tests:

Total protein	75	g/L	60 - 78
Albumin	**22 L**	g/L	35 - 52
Total bilirubin	**166 H**	umol/L	5 - 21
Conjugated bilirubin (DBil)	**85 H**	umol/L	0 - 3
Alanine transaminase (ALT)	**476 H**	U/L	7 - 35
Aspartate transaminase (AST)	**686 H**	U/L	13 - 35
Alkaline phosphatase (ALP)	**342 H**	U/L	42 - 98
Gamma-glutamyl transferase (GGT)	**41 H**	U/L	<40

Lipids:

Total cholesterol	1.34	mmol/L
Triglyceride	0.91	mmol/L
HDL cholesterol	0.13	mmol/L
Calculated LDLC (Sampson)	0.76	mmol/L
Calculated non-HDL cholesterol	1.21	mmol/L

CARDIOVASCULAR RISK

Lipid targets should be individualised per patient based on their cardiovascular risk category. This can be determined using an online or app-based risk calculator, e.g., the ESC app which provides access to several useful calculators.

https://www.escardio.org/Education/Practice-Tools/CVD-prevention/toolbox/SCORE-Risk-Charts

TREATMENT TARGETS FOR TOTAL CHOLESTEROL, LDLC, and non-HDLC:

Risk Category:	TC target:	LDLC target:	non-HDLC target:
Low Risk	<5.0 mmol/L	<3.0 mmol/L	<3.8 mmol/L
Moderate Risk	<4.6 mmol/L	<2.6 mmol/L	<3.4 mmol/L
High Risk	<4.0 mmol/L	<1.8 mmol/L	<2.6 mmol/L
Very High Risk	<3.6 mmol/L	<1.4 mmol/L	<2.2 mmol/L

Also, aim for a reduction in LDLC of >=50% from baseline for those in the high and very high risk categories. TC targets are not well defined but are provided as approximations when LDLC is not tested. Non-HDLC represents the total atherogenic lipoprotein burden and is particularly valuable in patients with elevated triglycerides, obesity, diabetes, and very low LDLC levels.

The Sampson equation shows superiority over earlier LDLC equations, particularly in patients with hypertriglyceridaemia or low LDLC, and works equally well on fasted and non-fasted samples. Calculated LDLC may be inaccurate when TG are >=9.04 mmol/L or there is variation in the composition of VLDL, such as in dysbetalipoproteinaemia,

Mabotsebane MATLALA

LAB NUMBER: **LH M0000621**
Collected: 17/03/2026 12:21
Printed: 19/03/2026 15:00

Bheki Mlangeni Hospital, Female Medical Ward, Hospital Number 11162673

Lipids:<Continued>

hyperlipoproteinaemia (a), or LpX.

Additionally, the following targets are considered favourable:
HDL Cholesterol 1.0 - 2.3 mmol/L
Fasting triglycerides <1.7 mmol/L
While higher HDLC levels are considered favourable, very low and very high HDLC levels are associated with increased cardiovascular risk.
When TG are >10 mmol/L there is increased risk for pancreatitis.

References:
Mach F, et al. 2019 ESC/EAS Guidelines for the management of dyslipidaemias: lipid modification to reduce cardiovascular risk. Eur Heart J. 2020 Jan 1;41(1):111-88. https://doi.org/10.1093/eurheartj/ehz455

Marais AD, et al. Management of hyperlipidaemia. S Afr Med J. 2024 Apr;114(4):e1016. https://doi.org/10.7196/SAMJ.2024.v114i4.1016

Inflammatory markers:

C-Reactive protein	7	mg/L	<10

Haematinics:

Vitamin B12	>1476	pmol/L

Thyroid function tests:

Thyroid stimulating hormone	14.27	mIU/L
Thyroxine (free T4)	10.8	pmol/L

Indices:

Indices in serum:

Haemoglobin index	2+
Bilirubin index	3+
Lipaemia index	Not detected

Authorised by: Nm Mthimunye (Medical Technologist) Lipids Vit B12 FT4
P Makau (Medical Technologist) Indices
R Mofokeng (Medical Technologist) Na K Cl Bicarb Urea Creat Ca Mg PO4 TP Alb T bili C bili ALT AST ALP GGT CRP TSH

-- End of Laboratory Report --

TrakCare Lab Web Results Viewer